

NIC CPD Micro-Credential: Catheter & Continence Care for Professional Caregivers

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Online duration: 5–6 CPD learning hours

Practical component: Separate supervised competency verification where the learner's role, training, care plan and employer policy permit it

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Essential boundary: Completion of this online CPD does **not**, by itself, authorise a caregiver to insert, remove, replace or independently manage urinary catheters or perform other invasive procedures. It is not a licence to practise catheterisation.

1. Course title

NIC CPD Micro-Credential: Catheter & Continence Care for Professional Caregivers

2. Course overview

This stand-alone CPD develops safe, dignified and person-centred continence support, toileting assistance, urinary-catheter care awareness, bowel-continence support, skin protection, documentation, communication and escalation.

The course applies infection-prevention principles specifically to catheter and continence care. It complements, rather than reproduces, NIC's Infection Prevention & Control, ADL & Personal Care, Safe Mobility & Falls Prevention, Dementia Care, Palliative Care, Wound Care, Nutrition and Documentation CPDs.

Current catheter guidance emphasises using urinary catheters only when clinically indicated, maintaining a closed drainage system where applicable, keeping unobstructed flow and using appropriate insertion and maintenance technique [1](#) [2](#) . Caregivers observe and report; they do not independently diagnose catheter-associated infection.

3. Target learners

Professional caregivers, healthcare assistants, nursing assistants, home-care workers, residential-care staff, hospital support workers, community caregivers, elder-care workers,

disability-support workers, caregivers supporting reduced mobility and existing care professionals seeking CPD.

Permitted responsibility depends on training, employer and facility policy, care plan, delegation, supervision, professional regulation and applicable Nigerian requirements. Not every caregiver is authorised to perform catheter procedures.

4. Entry expectations

Learners should understand basic caregiving, communication, personal care, infection prevention, dignity, documentation and escalation. No clinical qualification is required for online knowledge completion. Practical verification requires authorised supervision, suitable equipment and current local protocol.

5. CPD learning hours

The online component is designed for **5–6 CPD learning hours**, including lessons, visual learning, knowledge checks, scenarios, module assessments, final assessment and downloadable resources.

6. Central competency

Provide safe, respectful and person-centred continence support and appropriately assist clients with catheter-related care within an authorised scope of practice while protecting dignity, skin integrity, infection prevention, comfort and safety.

7. Learning outcomes

By completion, learners should be able to:

1. Define continence and incontinence.
2. Explain common urinary and bowel-incontinence causes.
3. Distinguish major urinary-incontinence types at awareness level.
4. Explain incontinence' s effect on dignity and quality of life.
5. Provide person-centred toileting support.
6. Explain appropriate continence products.
7. Explain basic urinary anatomy relevant to continence.
8. Identify common catheter and drainage systems.
9. Explain catheter purpose and maintenance principles.

10. Apply infection-prevention principles.
11. Position tubing and drainage bags appropriately within policy.
12. Recognise common catheter problems and red flags.
13. Understand catheter-associated infection awareness.
14. Support hygiene, comfort, privacy and consent.
15. Prevent and recognise incontinence-associated skin damage.
16. Support bowel continence and recognise constipation/diarrhoea concerns.
17. Document continence and catheter care objectively.
18. Explain caregiver scope and referral requirements.

8. Course structure

Module	Title	Lessons	Assessment
1	Understanding Continence and Incontinence	1. Continence; 2. Incontinence Types; 3. Causes and Impact	6 questions
2	Person-Centred Continence Support	4. The Support Framework; 5. Toileting and Products; 6. Dignity and Communication	6 questions
3	Urinary Catheters: Fundamentals	7. What Catheters Are; 8. Catheter and Bag Systems; 9. Scope and Indications	6 questions
4	Catheter Care and Infection Prevention	10. Maintenance Principles; 11. Drainage and Hygiene; 12. CAUTI Awareness	6 questions
5	Catheter Problems and Escalation	13. No Drainage and Leakage; 14. Blood, Pain and Dislodgement; 15. Stop–Protect–Escalate	6 questions
6	Bowel Continence, Constipation and Skin Protection	16. Bowel Support; 17. Constipation and	6 questions

		Diarrhoea; 18. IAD and Skin Care	
7	Dignity, Documentation and Professional Practice	19. Intimate Care; 20. Documentation and Handover; 21. Integrated Verification	6 questions

9. Complete Module 1 — Understanding Continence and Incontinence

Lesson 1. Continence

Continence is the ability to control bladder or bowel function sufficiently for the person's needs and circumstances. Continence is not simply the absence of accidents; it includes access, mobility, communication, privacy, suitable clothing, toileting support and respect.

Lesson 2. Incontinence Types

Urinary incontinence may be stress, urge, overflow, functional, mixed or transient. Bowel/faecal incontinence involves loss of bowel control; double incontinence refers to urinary and bowel incontinence together. These are awareness categories, not diagnoses for caregivers.

Lesson 3. Causes and Impact

Ageing does not make incontinence inevitable. Contributing factors may include reduced mobility, dementia, neurological conditions, diabetes, medication, infection, constipation, cognitive impairment, environmental barriers and limited toilet access.

Incontinence can cause embarrassment, anxiety, isolation, sleep disruption, loss of independence and caregiver burden. The response is person-centred support, observation, documentation and appropriate referral.

Scenario: A client has a new sudden change in continence. **Best response:** Document the change and escalate for assessment; do not dismiss it as ageing.

10. Complete Module 2 — Person-Centred Continence Support

Lesson 4. The Support Framework

Use:

Assess → Ask → Plan → Support → Promote Independence → Observe → Document → Escalate.

Assess the person's routine, mobility, communication, preferences and risks. Ask respectfully. Follow the plan. Support without taking over. Observe change and escalate.

Lesson 5. Toileting and Products

Support prompted or scheduled toileting only as part of an agreed plan. Respond to requests promptly, assist with safe mobility, clothing management, toilet access, hygiene and hand hygiene. Continence products should fit the plan and person's needs; they must not replace assessment, toileting opportunity or dignity.

Lesson 6. Dignity and Communication

Continence care is care of the person, not merely management of bodily functions.

Avoid shaming, infantilising adults, jokes, disgust reactions, unnecessary exposure and treating products as identity. Obtain consent, explain actions, protect privacy, respect gender preferences where feasible and consider Nigerian cultural expectations without stereotyping.

11. Complete Module 3 — Urinary Catheters: Fundamentals

Lesson 7. What Catheters Are

A urinary catheter is a tube used to drain urine. Types include indwelling/Foley catheters, intermittent catheterisation, suprapubic catheters and external/condom systems where relevant. The reason for use and duration should be clinically determined.

Lesson 8. Catheter and Bag Systems

Components may include catheter tubing, securement, connection, drainage bag, leg bag and night drainage bag. The system must be maintained according to the care plan, manufacturer instructions and facility policy.

Lesson 9. Scope and Indications

Insertion, removal, replacement, irrigation, sampling and other invasive procedures are not automatically within caregiver scope. Catheters should be used only when clinically indicated and managed according to professional direction ¹ .

Scenario: A relative asks a caregiver to remove a catheter at home. **Best response:** Do not remove it; refer to an authorised professional.

12. Complete Module 4 — Catheter Care and Infection Prevention

Lesson 10. Maintenance Principles

Perform hand hygiene and use PPE where appropriate. Maintain personal hygiene, avoid unnecessary disconnection, prevent kinks, keep drainage unobstructed, secure tubing according to policy and keep the bag below bladder level where appropriate. Do not pull, manipulate or flush unless trained, authorised and directed.

Lesson 11. Drainage and Hygiene

Keep the drainage outlet from contacting contaminated surfaces. Empty bags according to policy using clean technique, correct container and safe disposal. Record output only when required by the care plan. Protect privacy and explain each action.

Lesson 12. CAUTI Awareness

Warning signs may include fever, chills, new/worsening confusion in a relevant clinical context, lower abdominal discomfort, flank/back pain, blood in urine, reduced/absent output or systemic deterioration. Cloudy or foul-smelling urine alone does not prove infection.

Caregivers observe and report; they do not independently diagnose a catheter-associated infection.

13. Complete Module 5 — Catheter Problems and Escalation

Lesson 13. No Drainage and Leakage

No urine may relate to kinked tubing, bag position, a full bag, obstruction, displacement or another clinical cause. Safe first checks may include looking for visible kinks, ensuring the

bag is positioned correctly and checking whether it is full, but only within policy. Do not pull, flush or reposition invasively.

Leakage around the catheter may indicate obstruction, bladder spasm, displacement or another issue and requires professional direction.

Lesson 14. Blood, Pain and Dislodgement

Blood in urine, significant pain, sudden swelling, accidental removal, damaged/disconnected systems, reduced output or new distress require prompt escalation. Do not reinsert, replace or improvise.

Lesson 15. Stop–Protect–Escalate

Use:

Recognise → Check what is safely within your role → Protect the client → Escalate → Document.

For accidental removal, protect privacy, do not reinsert, seek urgent authorised guidance and record the event. For suspected blockage or systemic deterioration, escalate promptly.

14. Complete Module 6 — Bowel Continence, Constipation and Skin Protection

Lesson 16. Bowel Support

Provide private, respectful toileting assistance, hygiene, product changes and odour management. Observe bowel pattern and report changes. Never use humour or disgust to manage faecal incontinence.

Lesson 17. Constipation and Diarrhoea

Constipation awareness includes reduced bowel movements, hard stool, straining, abdominal discomfort, reduced appetite and behaviour change in vulnerable people. Risks include reduced mobility, low fluid intake, poor diet, medication, illness and routine change.

Diarrhoea includes increased frequency and loose/watery stool, with dehydration and infection-control risks. Do not prescribe medication; follow the care plan and escalate.

Lesson 18. IAD and Skin Care

Incontinence-associated dermatitis (IAD) is moisture-related skin damage from repeated exposure to urine and/or stool. Early signs may include redness, soreness, burning, rash or superficial breakdown. Prevent it through prompt changes, gentle cleansing, careful drying, barrier protection according to plan and reducing moisture exposure ³ .

IAD and pressure injury can overlap but are not identical. Caregivers should report changes rather than independently diagnose complex skin conditions.

15. Complete Module 7 — Dignity, Documentation and Professional Practice

Lesson 19. Intimate Care

Obtain consent, explain, protect privacy, expose only what is necessary, maintain professional boundaries, respect preferences and offer choices. Trauma-informed awareness means avoiding force, acknowledging distress and involving safeguarding pathways when needed.

If a client refuses, pause, listen, offer alternatives and escalate unresolved risk. If a client becomes angry or embarrassed, remain calm and respectful.

Lesson 20. Documentation and Handover

Record continence episodes, toileting assistance, product changes, skin observations, urine/stool observations where required, catheter observations, output when required, client response, problems, escalation and person notified.

Use objective language. For example:

“No urine observed in drainage bag over the observed period; visible tubing kink checked and nurse informed.”

Avoid “lazy,” “dirty,” “difficult” or other judgemental labels.

Lesson 21. Integrated Verification

Apply: **identify the person and plan; ask and obtain consent; support safely; promote independence; observe; protect skin and catheter system; document; hand over; escalate.** Online completion does not establish invasive catheter competence.

16. Visual learning plan

Visual	Purpose
Continence-support pathway	Explain Assess → Ask → Plan → Support → Independence → Observe → Document → Escalate
Urinary-system diagram	Explain awareness-level anatomy
Catheter-system diagram	Identify tubing, securement and drainage bag
Safe drainage illustration	Show bag position, unobstructed flow and outlet protection
CAUTI observation card	Show signs requiring reporting without diagnosis
Catheter-problem decision tree	Support safe first checks and escalation
Bowel-support pathway	Show dignity, hygiene, observation and referral
IAD versus pressure-awareness diagram	Support early reporting
Objective documentation comparison	Teach factual wording
Scope-of-practice decision tree	Separate knowledge from invasive competence

Visuals must be respectful, non-graphic, culturally appropriate and reviewed for safeguarding and dignity.

17. Scenario-based learning

1. **New continence change:** Record and escalate; do not dismiss as ageing.
2. **Refusal:** Respect refusal, offer options and escalate risk.
3. **Bag below floor level:** Correct only within policy and report if needed.
4. **No drainage:** Check visible kink/position/full bag within scope, then escalate.
5. **Cloudy urine:** Do not diagnose infection from one characteristic.
6. **Fever and reduced output:** Urgent professional escalation.
7. **Accidental removal:** Do not reinsert; protect and seek urgent guidance.
8. **Leakage:** Do not independently change or flush; escalate.
9. **IAD:** Prompt hygiene and barrier support according to plan; report skin change.

10. **Embarrassment:** Maintain privacy, calm communication and choice.

18. Module assessments

Module 1

1. Define continence. **Sufficient bladder/bowel control for the person's needs.**
2. Name three urinary incontinence types. **Stress, urge, overflow, functional, mixed or transient.**
3. Is incontinence inevitable with age? **No.**
4. Name three contributing factors. **Mobility, dementia, diabetes, medication, infection, constipation or barriers.**
5. Name three impacts. **Embarrassment, isolation, anxiety, sleep disruption or lost independence.**
6. What should sudden change trigger? **Observation, documentation and escalation.**

Module 2

1. State the support framework. **Assess, Ask, Plan, Support, Promote Independence, Observe, Document, Escalate.**
2. What should products not replace? **Toileting opportunity and person-centred assessment.**
3. Name three dignity actions. **Privacy, consent and appropriate exposure.**
4. What should be avoided? **Shaming, jokes and disgust reactions.**
5. What supports independence? **Choice, prompt response and appropriate assistance.**
6. What if care is refused? **Pause, listen, offer choice and escalate risk.**

Module 3

1. What is a urinary catheter? **A tube used to drain urine.**
2. Name three catheter systems. **Indwelling, suprapubic, intermittent or external.**
3. Is insertion automatically within caregiver scope? **No.**
4. Who determines clinical indication? **Appropriately authorised clinical professional.**
5. What must guide management? **Care plan, policy, supervision and professional direction.**
6. What should a caregiver do when asked to remove one? **Refer if unauthorised.**

Module 4

1. Name four maintenance principles. **Hand hygiene, avoid disconnection, prevent kinks, unobstructed flow and bag positioning.**
2. Where should the bag generally be? **Below bladder level where appropriate.**
3. Should outlet touch contaminated surfaces? **No.**
4. Is cloudy urine alone proof of infection? **No.**
5. Who diagnoses CAUTI? **Authorised healthcare professional.**
6. What should output follow? **Care plan and policy.**

Module 5

1. Name three causes of no drainage. **Kink, position, full bag, obstruction or displacement.**
2. What may leakage require? **Professional assessment.**
3. What if catheter is accidentally removed? **Do not reinsert; escalate urgently.**
4. What if blood appears in urine? **Assess urgency and escalate.**
5. State the response framework. **Recognise, safe check, protect, escalate, document.**
6. Should caregivers flush a blocked catheter? **Not unless specifically trained, authorised and directed.**

Module 6

1. Name constipation signs. **Hard stool, straining, reduced movements, discomfort or appetite change.**
2. Name diarrhoea risks. **Dehydration and infection transmission.**
3. Can caregivers prescribe medication? **No.**
4. What is IAD? **Moisture-related skin damage from urine/stool exposure.**
5. Name two prevention actions. **Prompt changes, gentle cleansing, drying and barrier protection according to plan.**
6. Should complex skin conditions be diagnosed independently? **No.**

Module 7

1. Name four intimate-care safeguards. **Consent, privacy, dignity and appropriate exposure.**

2. What should documentation contain? **Episode, care, observation, response, problems, escalation and notification.**
3. What language should be used? **Objective and factual language.**
4. Does documentation replace escalation? **No.**
5. Does online completion authorise catheter insertion? **No.**
6. What is the integrated goal? **Safe, dignified support and timely escalation.**

19. 40-question final assessment

Recommended pass mark: 80% (32/40). At least 30 questions are scenario/application based.

1. **MCQ:** Continence means: A) no need for support; B) control appropriate to the person's needs; C) catheter use; D) a diagnosis. **Answer: B.**
2. **Scenario:** New incontinence in an older adult. **Answer:** Observe, document and escalate; do not dismiss as ageing.
3. **True/False:** Products should replace regular toileting support. **Answer: False.**
4. **Scenario:** Client embarrassed after an accident. **Answer:** Protect privacy, reassure and provide respectful care.
5. **Application:** Name three causes of incontinence. **Answer:** Mobility, dementia, diabetes, medication, infection, constipation or environmental barriers.
6. **Scenario:** Client requests toilet assistance. **Answer:** Respond promptly and support safely.
7. **MCQ:** Functional incontinence may relate to: A) access or mobility barriers; B) only infection; C) catheter insertion; D) diagnosis. **Answer: A.**
8. **What should not happen?** Joke about incontinence.
9. **MCQ:** A urinary catheter is: A) a drainage tube; B) a dressing; C) a medication; D) a diagnosis. **Answer: A.**
10. **Scenario:** Asked to remove Foley catheter. **Answer:** Do not remove unless authorised/trained; refer.
11. **True/False:** Catheter insertion and removal are automatically within all caregiver roles. **Answer: False.**
12. **Application:** Name four catheter system components. **Answer:** Tubing, securement, connection, drainage bag, leg/night bag.

13. **Scenario:** Tubing is kinked and bag is full. **Answer:** Correct safe visible issues within policy and escalate if output does not resume.
14. **MCQ:** The drainage bag should generally be: A) above head; B) below bladder level where appropriate; C) on floor; D) disconnected. **Answer: B.**
15. **Scenario:** Outlet touches floor. **Answer:** Protect system and follow local policy; report contamination.
16. **True/False:** Unnecessary disconnection increases risk. **Answer: True.**
17. **Scenario:** Cloudy urine but no other change. **Answer:** Do not diagnose; observe and report according to plan.
18. **Application:** Name four possible CAUTI warning signs. **Answer:** Fever, chills, pain, systemic deterioration, reduced output, blood or relevant new confusion.
19. **Scenario:** Fever and flank pain. **Answer:** Prompt professional escalation.
20. **What should not happen?** Start antibiotics independently.
21. **Scenario:** No drainage despite safe visible checks. **Answer:** Protect client and escalate; do not flush or reinsert.
22. **MCQ:** Leakage may indicate: A) only normal function; B) obstruction, displacement, spasm or other cause; C) permission to remove; D) no concern. **Answer: B.**
23. **Scenario:** Catheter accidentally comes out. **Answer:** Do not reinsert; seek urgent authorised guidance.
24. **True/False:** Blood in urine can require prompt escalation. **Answer: True.**
25. **Scenario:** Client has severe catheter pain. **Answer:** Stop manipulation, protect and escalate.
26. **Application:** State the catheter-problem framework. **Answer:** Recognise, safe check, protect, escalate, document.
27. **Scenario:** Constipation with abdominal discomfort and reduced appetite. **Answer:** Document and escalate; do not prescribe laxative.
28. **MCQ:** Diarrhoea increases concern for: A) dehydration and infection spread; B) guaranteed recovery; C) no risk; D) catheter authorisation. **Answer: A.**
29. **Scenario:** Moist, sore skin after repeated incontinence. **Answer:** Gentle care/barrier according to plan and report possible IAD.
30. **True/False:** IAD and pressure injury are identical. **Answer: False.**
31. **Scenario:** Client refuses intimate care. **Answer:** Respect, listen, offer choices and escalate unresolved risk.
32. **What should not happen?** Expose more of the body than necessary.

33. **Scenario:** Caregiver writes “dirty and difficult.” **Answer:** Replace with objective observations.
34. **Application:** Name five documentation fields. **Answer:** Episode, assistance, product change, skin/catheter observation, response, problem, escalation and notification.
35. **Scenario:** Output is required by the care plan but is much lower than usual. **Answer:** Record and promptly escalate.
36. **True/False:** Documentation replaces clinical escalation. **Answer: False.**
37. **Scenario:** Family instruction conflicts with care plan. **Answer:** Follow authorised plan and escalate conflict.
38. **MCQ:** Practical competence requires: A) online reading; B) supervised assessment; C) attendance only; D) confidence. **Answer: B.**
39. **Scenario:** Caregiver is unsure whether to flush a catheter. **Answer:** Do not improvise; seek authorised direction.
40. **Integrated scenario:** A client with a catheter has reduced output, new lower abdominal discomfort and embarrassment. **Answer:** Preserve dignity, check only safe visible issues within policy, do not manipulate invasively, escalate promptly and document objectively.

20. Answer key

Answers are embedded in the assessment. The governing principles are: continence care is person-centred; identity, consent, privacy and dignity matter; catheter systems must be protected from obstruction and contamination; caregivers observe and report rather than diagnose; invasive procedures require authorisation and supervision; and uncertainty is a reason to escalate rather than improvise.

21. Practical competency verification framework

Practical verification is separate from online completion and must be limited to skills authorised by the employer/facility and applicable requirements.

Station	Capability
Continence support	Privacy, consent, toileting, clothing, hygiene and independence
Product support	Appropriate product use according to plan and skin protection

Catheter-system observation	Identifies tubing, bag, securement, kinks and drainage concerns
Drainage-bag support	Demonstrates approved emptying/handling procedure under supervision
Infection prevention	Hand hygiene, PPE, clean technique and contamination prevention
Problem response	Recognises no drainage, leakage, pain, blood and dislodgement
Escalation	Uses correct contacts and communicates urgency
Documentation	Records objective observation, action, response and notification
Safeguarding	Maintains intimate-care boundaries and responds to distress/refusal

Critical failures: inserting/removing/replacing without authority; flushing or irrigating without authorisation; ignoring reduced output or severe pain; contaminating the system; breaking privacy; coercing intimate care; failing to report a dislodgement or exposure; or signing off an unobserved skill.

22. Downloadable resources

1. Continence Support Quick Reference.
2. Toileting and Prompted-Toileting Checklist.
3. Catheter-System Observation Guide.
4. Catheter Hygiene and Drainage-Bag Checklist.
5. Catheter Problem Troubleshooting Guide.
6. CAUTI Awareness and Escalation Card.
7. Bowel Continence Support Guide.
8. Constipation and Diarrhoea Observation Guide.
9. IAD and Skin Protection Checklist.
10. Continence Documentation Template.
11. Intimate-Care Communication Guide.
12. Practical Competency Checklist.

23. Certificate recommendation

Online CPD certificate

This certificate recognises successful completion of NIC online CPD in continence and catheter-care awareness, including person-centred toileting, continence products, catheter systems, maintenance principles, infection prevention, drainage awareness, common problems, bowel continence, skin protection, dignity, documentation and escalation. **This certificate represents online knowledge completion only. It does not authorise catheter insertion, removal, replacement, flushing, irrigation or other invasive procedures. It is not a nursing, medical or continence-specialist qualification, professional licence or automatic statutory workplace certification.**

Practical endorsement

Issued only after supervised assessment of named skills by an authorised assessor, with date, equipment, local protocol, result, remediation and reassessment. It does not expand the learner's legal or employment scope.

24. Digital badge recommendation

NIC Catheter & Continence Care CPD — Knowledge Completion

Field	Specification
Description	Online CPD knowledge in dignified continence and catheter-care support
Competencies	Continence, toileting, catheter awareness, infection prevention, skin protection, escalation and documentation
CPD hours	5–6 online hours
Evidence	Modules, assessments, final score $\geq 80\%$, scope acknowledgement
Limitation	No invasive-procedure authorisation or professional licence
Review	Recommend review every 24 months or when guidance changes

25. Portal implementation notes

Use **Course → Modules → Lessons → Knowledge Checks → Module Assessments → Final Assessment → Certificate**. Track online completion, score, scope acknowledgement, practical verification status, assessor evidence, badge status and review date separately.

26. Nigerian context and scope-of-practice notes

Hospitals, clinics, residential care, home care, community care and family-supported environments differ in supplies, privacy, water, sanitation, staffing, referral access and supervision. NIC should require each employer to insert current catheter SOPs, drainage-bag procedure, output chart, infection pathway, emergency contacts, safeguarding route, waste procedure and authorised caregiver activities.

Do not invent Nigerian legal or regulatory requirements. Verify any Nigerian scope, delegation or reporting claim against current authoritative Nigerian sources and the employer's governance process before publication.

27. References

- [1] Centers for Disease Control and Prevention, Preventing Catheter-associated Urinary Tract Infections.
- [2] Centers for Disease Control and Prevention, Catheter-Associated Urinary Tract Infections.
- [3] Kottner et al., Incontinence-Associated Dermatitis in Older Adults.
- [4] NICE, Urinary incontinence in women: quality standard and continence-care principles.
- [5] World Health Organization, WHO Guidelines on Hand Hygiene in Health Care.

28. Quality control review

Review area	Finding	Required action
Clinical accuracy	Continence, catheter systems, drainage, bowel care and skin protection are covered at caregiver level.	Clinical and continence-specialist review.
Patient safety	Invasive boundaries, CAUTI awareness and catheter red flags are prominent.	Add facility-specific emergency contacts.

Dignity	Intimate-care privacy, consent, communication and safeguarding are central.	Safeguarding review and learner pilot.
Scope	Insertion, removal, replacement, flushing and irrigation are not authorised by online completion.	Governance sign-off.
Nigerian relevance	Contextual variation and local SOP insertion are acknowledged.	Verify all local claims.
Assessments	40 questions with scenario emphasis are included.	Pilot and item-analyse.
Practical competence	Separate framework and critical failures are provided.	Approve assessors and equipment.
Duplication	Links to related NIC CPDs are explicit.	Add portal cross-links.
Accessibility	Plain language, tables, scenarios and job aids are used.	Test with varied learners.
Visual quality	Visual plan is respectful and non-graphic.	Review all images before publication.

Final learner message: “I can support continence and catheter care professionally, protect dignity and safety, recognise problems early, and know when to escalate.”